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Frank A. Beach, *Editor*
University of California, Berkeley

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From Child Sexual Abuse to Adult Sexual Risk

Trauma, Revictimization, and Intervention

Edited by
Linda J. Koenig, Lynda S. Doll,
Ann O'Leary, and Willo Pequegnat

CHILD SEXUAL ABUSE AND ADULT SEXUAL RISK: WHERE ARE WE NOW?

LYNDA S. DOLL, LINDA J. KOENIG, AND DAVID W. PURCELL

Child sexual abuse (CSA), while not a new problem, has only received significant attention in both academic and public arenas within the last two decades. Heightened awareness of the problem may stem from a growing societal concern over child maltreatment in general, an increased comfort level in discussing sexual violence, or perhaps even the publicity surrounding several recent cases. Whatever the source, this attention has been accompanied by a growing recognition that children who experience sexual violence may be vulnerable to many negative life outcomes in the years following abuse.

Much of the research about abuse sequelae has addressed psychiatric morbidity and mental health. However, an emerging body of literature suggests that children who are abused also face increased risk of physical health problems in adulthood, including negative sexual and reproductive health consequences such as unwanted pregnancy, sexually transmitted diseases including HIV infection, and adult sexual violence. In this book, we bridge the current scientific literature about CSA, basic trauma research, and clinical practice to help practitioners and researchers meet the needs of this vulnerable population. We review evidence about the link between CSA and adult outcomes and draw on both theory and data to begin translating that knowledge into interventions for people who have experienced CSA.

This chapter was authored or coauthored by an employee of the United States government as part of official duty and is considered to be in the public domain. Any views expressed herein do not necessarily represent the views of the United States government.

PREVALENCE OF CHILD SEXUAL ABUSE

Considerable effort has recently gone into determining the number of children who have experienced CSA (Haugaard, 2000). Even the most conservative estimates from these studies indicate that these rates have reached alarming proportions in the United States.

Overall, data from large or nationally representative samples suggest that 12% to 53% of girls and 3% to 16% of boys experience some form of sexual abuse during childhood (Finkelhor, Hotaling, Lewis, & Smith, 1990; Holmes & Slap, 1998; Molnar, Buka, & Kessler, 2001; Rind & Tromovitch, 1997). Rates appear higher when samples are gathered from convenience or other selected populations. For example, in their report, Holmes and Slap (1998) noted that prevalence rates among boys vary dramatically from 4% to 76% across studies. In a systematic review of data derived from 16 U.S. and Canadian studies published over 25 years, Gorey and Leslie (1997) estimated unadjusted prevalence rates of 23% among girls and 9% among boys across all studies. Twelve of the studies used random samples derived from the general population; four used convenience samples from captive audiences such as college students. More than two thirds of the aggregate sample from all studies was female.

Prevalence rates for girls are usually three to five times higher than for boys. However, cases among boys may be less likely to be reported, perhaps because such cases usually involve same-sex behavior that is more stigmatized by society (Boney-McCoy & Finkelhor, 1995). Sexual orientation is a key variable to consider when examining retrospective reports of CSA among adult men. Generally, men who have sex with men report a much higher prevalence than do heterosexual men (Doll et al., 1992; Lenderking et al., 1997; Paul, Catania, Pollack, & Stall, 2001). Rates typically range from 20% to 39%, similar to or only slightly higher than those reported in studies of general population women. Whereas most of the research showing elevated prevalence rates among men who have sex with men has used convenience samples, work by Paul et al. (2001) that used a more representative sample of this population recruited in four urban areas found similarly elevated rates. With a relatively stringent definition that included actual sexual contact as well as coercion, the prevalence rate was 21%.

Few studies have examined racial or ethnic differences in prevalence rates. Among those that have, some suggest that CSA is equally prevalent among Black and White girls (Cappelleri, Eckenrode, & Powers, 1993; Wyatt, 1985; Wyatt, Loeb, Solis, & Carmona, 1999); some studies show lower rates for girls from other racial and ethnic backgrounds (Cappelleri et al., 1993), whereas others show no differences (Romero, Wyatt, Loeb, Carmona, & Solis, 1999). In their review, Holmes and Slap (1998) found that more men of color reported experiencing CSA than did White men. Additional data suggest that gay men of color may be particularly likely to report such experiences during their childhood (Carballo-Diéguez & Dolezal, 1995; Doll et al., 1992).

Prevalence rates may also differ depending on the methodological rigor and the definition used for CSA in the studies. Gorey and Leslie (1997) estimated that prevalence rates were inversely associated with participant response rates. In the aggregated female sample derived from the studies Gorey and Leslie reviewed, estimated prevalence rates were two thirds higher (27.8%) in studies with responses rates below 60% than in studies with response rates of 60% or more (16.8%). Estimated rates were 15% and 7% for girls and boys, respectively, when a broad, noncontact definition of abuse was excluded from the calculations. After excluding noncontact abuse from the definition and taking into account the effect of differences in response rates, Gorey and Leslie provided general population prevalence estimates of 12% to 17% for girls and 5% to 8% for boys.

ADULT SEQUELAE OF CHILD SEXUAL ABUSE

The number of children experiencing sexual violence during childhood or adolescence is clearly alarming. Primary prevention efforts that focus on child safety, family functioning, and the characteristics of perpetrators must be enhanced to address these disturbing figures. Also important, and the primary focus of this book, is the number of children and adults who experience heightened vulnerability to a range of negative life outcomes, linked in part to these early sexual experiences. Increased rates of sexual revictimization (e.g., rape), sexual risk (e.g., multiple sexual partners, commercial sex work), and other forms of maltreatment (e.g., physical assault, crime victimization) have been found among people reporting CSA, as compared with those who have not reported such experiences (see chap. 2, this volume; Tjaden & Thoennes, 2000).

Moreover, research has consistently shown that CSA is one of the strongest predictors of adult sexual assault (chap. 2, this volume; Coid et al., 2001). The National Violence Against Women Survey found that 18% of the women raped before age 18 were also raped after the age of 18, compared with only 9% of women who did not report being raped when they were young (Tjaden & Thoennes, 2000). Data from 8,000 men ages 18 and older collected during this same survey showed that men who reported being sexually abused as children were nearly five times more likely to be sexually revictimized by a nonintimate partner than were those who did not report being sexually abused (Desai, Arias, Thompson, & Basile, 2002). Similarly, in a study of college students, Gidycz and colleagues (Gidycz, Coble, Latham, & Layman, 1993) found that 30% of female victims of childhood rape and 32% of female victims of attempted childhood rape were revictimized in adulthood. By com-

parison, only 14% of women with no history of CSA reported being raped during adulthood.

Also disturbing are the high rates of negative psychological, reproductive, and other health outcomes experienced by those reporting CSA. These negative outcomes include posttraumatic stress symptoms, drug abuse and dependency, unintended pregnancy, gynecological complications, and sexually transmitted diseases such as HIV infection (Bartholow et al., 1994; Bensley, Van Eenwyk, & Simmons, 2000; Doll et al., 1992; Molnar et al., 2001; Thompson, Arias, Basile, & Desai, 2002; see also chaps. 1 & 8, this volume). Such data confirm the argument that child, adolescent, and adult victims of sexual violence urgently need greater medical, psychological, and public health attention.

INTRODUCTION TO THE BOOK

The first goal of this book is to synthesize and bring attention to the growing scientific literature about sequelae of CSA, particularly sexual revictimization (e.g., experiencing sexual violence or rape with another person) and sexual risk (e.g., having multiple sexual partners, unprotected sexual intercourse). Toward this end, we bring together research from a variety of disciplines, including epidemiology, traumatology, and prevention science, to better understand the relationship between CSA and adult sexual risk and to explore opportunities for prevention. The book contains information that will be helpful to those working with people who have a history of CSA; however, overall, the book does not concentrate on clinical applications.

The book has three major parts, as well as an introduction and conclusion. Part II, Child Sexual Abuse and Sexual Risk in Adulthood, provides a foundation for the remaining two sections of the book by summarizing research about the relationship between CSA and a range of social, psychological, behavioral, and health outcomes. In chapter 1, Heiman and Heard-Davison summarize the association between CSA and adult sexual functioning and relationship problems. In chapter 2, Rich, Combs-Lane, Resnick, and Kilpatrick assess the complex relationship between CSA and adult sexual violence or revictimization. In chapter 3, Koenig and Clark explore the link between CSA and HIV infection in women. Finally, in chapter 4, Purcell, Malow, Dolezal, and Carballo-Diéguez describe the link between CSA of boys and adult functioning, including sexual risk behaviors.

Part III, Theoretical Bases for Adult Risk and Revictimization, describes cognitive, social, and behavioral processes that may mediate and potentially mitigate the relation between these child experiences and adult outcomes. Quina, Morokoff, Harlow, and Zurbriggen begin this section in chapter 5 with a focus on cognitive and attitudinal consequences of abuse and their role in adult risk behavior. In chapter 6, Zurbriggen and Freyd take a cognitive processing approach to the relation between CSA and adult sexual risk, focusing on the role of dissociative tendencies, information-processing effects, and consensual sex decision mechanisms. In chapter 7, Lynn, Pintar, Fite, Ecklund, and Stafford describe a social narrative model in which they argue that the key to revictimization is the social contexts in which the children lived and in which they continue to live as adults. In chapter 8, Wilsnack, Wilsnack, Kristjanson, Vogeltanz-Holm, and Harris explore alcohol use as a mediator between CSA and adult sexual risk. The section ends with chapter 9, by Pennebaker and Stone, who describe the importance of linguistic processing, such as occurs when traumatic experiences are translated into words and stories, in managing the effect associated with traumatic events such as CSA.

Part IV, Interventions to Promote Healthier Sexual Outcomes Among Child Sexual Abuse Survivors, describes a variety of therapeutic issues and interventions for adults. In chapter 10, Briere describes how HIV prevention can be incorporated into traditional psychotherapeutic treatment of sexual abuse trauma. Chin, Wyatt, Vargas Carmona, Loeb, and Myers then describe in chapter 11 preliminary qualitative findings from an evaluation of an integrative risk-reduction approach for HIV-seropositive women with CSA histories. In chapter 12, Spiegel, Classen, Thurston, and Butler describe two psychotherapeutic models that may ameliorate negative sequelae—one that focuses on memories of past abuse and another that focuses on present emotional and interpersonal experiences. Finally, in chapter 13, Blackwell, Lynn, Vanderhoff, and Gidycz review outcomes of sexual assault risk reduction programs, both for general population women and for women with histories of CSA. This book concludes with a chapter by O'Leary, Koenig, and Doll identifying future research directions.

As the book was being planned, the editors made several decisions about its focus. For example, until recently, research has generally investigated sexual abuse of girls. While researchers have broadened their work to include sexual abuse of boys, many of these studies have lacked methodological rigor (Holmes & Slap, 1998; Rind, Tromovitch, & Bauserman, 1998). Even today, some studies continue to combine data on girls and boys for analysis, despite growing evidence that gender differences may be particularly important in assessing outcomes (Rind et al., 1998). In this book we include a separate chapter about CSA and its sequelae among men and discuss limited data on men throughout other chapters. However, given the state of the science, this book emphasizes experiences of women, among whom prevalence rates are higher (Djeddah, Facchin, Ranzato, & Romer, 2000; chap. 2, this volume).

The body of well-designed studies on sexual abuse is growing. However, as readers will note throughout the book, the authors struggled with the variety of terms and methods used to assess the experiences (Goldman & Padayachi, 2000). For example, little consensus exists about how to define sexual abuse (Haugaard, 2000; Holmes & Slap, 1998; Neumann, Houskamp,

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This remarkable volume successfully synthesizes diverse areas of study to create an for researchers, clinicians, and educators. Using solid empirical data, the contributors separate myth from scientific evidence in this oftentimes murky area. This is a "must-have" in your library that will be used for many years.

—Eli Coleman, PhD, Professor and Director, Program in Human Sexuality, University of Minnesota Medical School, Minneapolis

This well-written book provides a wealth of clinically relevant scientific information about the far-reaching adult sequelae of child sexual abuse. By bringing together findings from epidemiology, traumatology, and prevention science, this book makes a significant and unique contribution toward assisting practitioners and researchers in better understanding child sexual abuse and its impact into adulthood. The chapter authors present an impressive integration of theory and research that will benefit mental health professionals in their work with female and male victims of sexual violence.

—Nadine J. Kaslow, PhD, ABPP, Department of Psychiatry and Behavioral Sciences, Emory University School of Medicine, Atlanta, GA

As a nurse, I know that other nurses who work in this area will find the book useful. As a professor of public health, I also know that public health professionals who work in behavior or prevention sciences will find the book a valuable resource. Health educators who often provide programs within large public health settings will benefit from the book as well. I think the book is timely, and researchers will appreciate the work of the editors and authors in publishing this material.

—Colleen DiIorio, PhD, RN, FAAN, Professor, Department of Behavioral Sciences and Health Education, Rollins School of Public Health, Emory University, Atlanta, GA

Also of Interest From APA Books

Health Consequences of Abuse in the Family: A Clinical Guide for Evidence-Based Practice
Edited by Kathleen A. Kendall-Tackett

Health Consequences of Abuse in the Family translates research into practice by examining the intersection of family violence and health. Specifically, this volume looks at the health care needs of people who have experienced abuse and subsequently have related chronic diseases and conditions. Chapters address the implications for clinical practice as well as review relevant studies and provide additional resources. The special needs of children with disabilities, elders, and women are also discussed.

Domestic Violence in the Lives of Children: The Future of Research, Intervention, and Social Policy
Edited by Sandra A. Graham-Bermann and Jeffrey L. Edleson

Clinicians know from experience that some children who have been exposed to domestic violence are severely affected by the experience, whereas others appear relatively unscathed. Much less clear is what protects children in the latter group, and how the larger community and social environment impedes or enhances adjustment in their families. Cutting-edge methodology is emphasized, with the contributors discussing important research constructs, the best available measures and methods of analyzing data, the effectiveness of interventions, and implications for public policy. This book will be invaluable to researchers, policymakers, and all those who care about improving our understanding of the impact of domestic violence on children.

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FROM CHILD SEXUAL ABUSE
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BITCH

ON THE FEMALE OF THE SPECIES

LUCY COOKE

AUTHOR'S NOTE ON LANGUAGE

Language evolves rapidly, and there is currently much conversation about the conflation of sex and gender terms. It is critical to use these terms appropriately and not to confuse them. Most scientists agree that non-human animals do not have gender. In this book, the terms female and male refer to an animal's biological sex. I do engage in anthropomorphizing, to an extent. Sometimes this is because these were the historical terms used. For example, I may refer to an animal's genitalia as being 'masculinized' or a brain being 'feminized' as this was the original scientific description. Such gendered terms needn't and shouldn't be used to describe animals' sex characteristics and behaviours in scholarly realms today. I also use gendered terms such as 'mother' and 'father' to describe animals, because these are the terms used by the scientists in question and most of my audience will understand what or who I refer to with these terms – for instance, 'mother' may mean the egg-producing parent of an individual animal. At other times, I have used anthropomorphic terms such as femme fatale, queen, lesbian, sister, lady and bitch for storytelling purposes, and readers needn't choose to replicate these labels in their academic work. I recognize that this anthropomorphizing can, unintentionally, have gendered implications. This book intends to demonstrate that sex is wildly variable and that gendered ideas based on assumptions of binary sex are nonsense. It is my sincerest hope that this intent has been clearly communicated.

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AND OTHER TALES OF HUMAN SEXUALITY

JOSEPH GLENMULLEN, M.D.

FOREWORD BY ROBERT COLES

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Over and over again in psychotherapy," says Dr. Joseph Glenmullen, a Harvard psychiatrist, "one sees that sexual issues and symptoms are metaphors for the larger psychological stresses and strains in an individual's life." In this engaging collection of cases, he shows how sex is a microcosm of interpersonal relationships, a shorthand in which an individual's psychological makeup can often be seen in bold relief.

Dr. Glenmullen observes that the twentieth century is unique in its sweeping cultural shifts in attitudes toward sexuality. From Victorian prudery at the turn of the century, the pendulum swung to the unbridled sexual liberty of the 1960s and back again to the conservatism of the last decade. While these changes have been dazzling in their swiftness and breadth, all too often in their wake they have left people confused and conflicted.

Such confusion and conflicts are the subject of *The Pornographer's Grief and Other Tales of Human Sexuality*. In the title case, Scott cannot overcome his addiction to buying pornography until he uncovers its surprising symbolic meaning. In "The Woman Who Thought Her Orgasm Was a Gift," when Anne tries to unravel her mental block to achieving orgasm, she discovers a tangled knot of vulnerability, devalued images of womanhood, and the power struggle between the sexes. And in "Don Juan's Regret," Rick is a compulsive womanizer who is dismayed to find what underlies his lack of depth and commitment in relationships.

These intimate and thought-provoking cases cover a remarkable range of themes including addictive sexual behavior, sexual dysfunction, and the influence of drugs, trauma, and repressive religious backgrounds on human sexuality. Equally illuminating about the process of psychotherapy, each

(continued on back flap)

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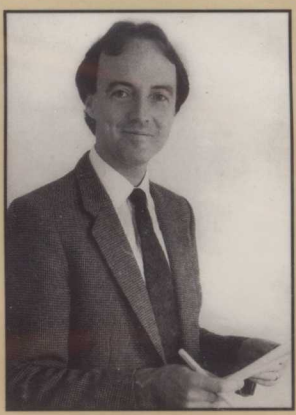
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one is an archaeological journey through the many layers of an individual's experience to discover unconscious motivations and long-forgotten influences from the past. Dr. Glenmullen comes to some startling conclusions not only about the individuals he treats but about sexual behavior in general. Told in a lively, anecdotal style, these fascinating reflections on our times will captivate anyone interested in the complex web of human sexuality.



JOSEPH GLENMULLEN, M.D., is an instructor in psychiatry at the Harvard Medical School, on the staff of the Harvard University Health Services, and in private practice in Cambridge, Massachusetts. He lives with his wife and children in Cambridge.

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